Shift Report

Ridgeline Regional Medical Center · Tier 1 Healthcare IT Support · Simulation (synthetic data)

Agent name	Dennis Lam
Date	Monday, August 17, 2026
Shift	Monday day shift: signed on 7:00 AM; queue assigned from the on-call rotation at 8:45 AM; cleared and documented by 11:00 AM

1. Ticket Triage Log

Queue scan at 8:45 AM, after the last ticket (T-1046) was logged at 8:41 AM. SLA remaining is calculated from each open timestamp (P1 30 min, P2 2 h, P3 8 h); no ticket had a valid pause. T-1045 was within one hour of breach and moved to the top; T-1043 had already breached and came next. Tickets are listed in working order.

Ticket ID	Category	Priority	SLA Remaining (at 8:45 AM)	Disposition Decision
T-1045	Network	P1	4min	Closed
T-1043	Hardware	P1	0min (breached 40min ago; due 8:05 AM)	Escalated
T-1044	Software	P2	1h 17min	Continue Troubleshooting
T-1046	Software	P2	1h 56min	Continue Troubleshooting
T-1042	Access	P3	6h 27min	Fulfilled

Triage notes

- **T-1045 (opened 8:19 AM, due 8:49 AM):** P1: losing nurse call in a care area is a direct patient-safety risk. Network: several east-wing units failed at once; on-call called the NOC line at 8:22 AM.
- **T-1043 (opened 7:35 AM, due 8:05 AM):** P1: an infusion pump alert fault on a device in patient use matches the reference example. Hardware (medical device). No pause was recorded, and none would be valid without a documented safety check.
- **T-1044 (opened 8:02 AM, due 10:02 AM):** A pre-contrast creatinine result is missing from a CT order. P2 ("lab results not syncing"; workaround available). Software (results interface).
- **T-1046 (opened 8:41 AM, due 10:41 AM):** One provider's prescriptions failing to e-fax to a pharmacy. P2 ("e-prescribing failing for a single provider"). Software (e-fax server).
- **T-1042 (opened 7:12 AM, due 3:12 PM):** Patient portal reset emails not arriving. P3 (one user, non-clinical system). Access; a standard request, pre-approved once identity is verified.

Closure note: T-1045

Closed	8:47 AM; SLA met with 2min remaining.
End-user confirmation	At 8:46 AM the east wing charge nurse confirmed nurse call had worked normally since 8:31 AM; her 8:40 AM test calls from three rooms reached the station console and nurses' wireless phones.
Infrastructure confirmation	Network Operations: an east-wing IDF switch uplink flapped 8:10–8:31 AM and recovered on its own, no alarms since. NOC agreed to closure and is tracking it as a problem record.
Reopen criteria	Any recurrence: reopen as P1 and call the NOC line immediately.

2. Fulfillment Records

Ticket ID	T-1042: Patient portal password reset not delivering (P3, Access)
Action Taken	• **10:15 AM:** Called the patient back and verified identity per portal procedure (name, date of birth, address). • **10:18 AM:** Patient portal administration console: account active, not locked; the email on file had a domain typo ("@gmial.com"), so all four weekend reset emails bounced. • **10:21 AM:** Corrected the email address with the patient's verbal confirmation. • **10:23 AM:** Sent a new reset link; the patient set a new password and logged in at 10:27 AM while on the phone.
Quantities	1 email corrected; 1 password reset issued; 1 login confirmed.
Exceptions	None.
Confirmation of documentation back into the ticket	At 10:32 AM I updated T-1042 with the root cause, each timestamped action, the identity-verification method (identifier types only) and the patient's login confirmation. Status set to Fulfilled at 10:34 AM; SLA met with 4h 38min remaining.

3. Troubleshooting Notes

T-1044: Lab results not syncing to EHR for radiology order (P2, Software)

Main-campus CT reported a pre-contrast creatinine/eGFR result missing from the EHR for a scheduled contrast CT. Worked 9:05–9:30 AM.

#	Step attempted (system / tool checked)	Result
1	EHR Results Review (user signed out, back in and refreshed).	Result missing from chart and CT order; not a display or cache issue.
2	Laboratory Information System (LIS) read-only inquiry.	Final-verified 7:31 AM. The failure is in the LIS-to-EHR transfer.
3	IT change calendar.	Interface engine maintenance ran 6:00–7:45 AM; the result was finalized during it.
4	Interface engine dashboard (read-only), LIS-to-EHR results interface.	Up and replaying held messages (queue 46 → 39, 9:10–9:25 AM). This result is Pending, not Error, so no Tier 1 reprocessing is needed.
5	Asked the requester for other affected orders.	Possibly several CT patients; accession numbers not available.

Workaround: the lab phoned the result to CT at 8:20 AM (read-back documented); the scan went ahead.

SLA status: follow-up for accession numbers sent at 9:30 AM; clock paused with 32min remaining (due 10:02 AM). No reply by end of shift.

Recommended next step for the incoming technician: when radiology replies, the clock resumes with 32min. Check each accession in the dashboard and EHR. If all have posted, confirm with the requester and close. If any message shows Error or the queue stops draining, escalate to Clinical Informatics by ticket with the accession list and queue counts, adding a patient-safety flag if a scan is delayed.

T-1046: Prescription e-fax failing to transmit to pharmacy (P2, Software)

An outpatient clinic reported one provider's e-faxed prescriptions failing to reach a community pharmacy since about 8:30 AM. Worked 9:35–10:12 AM.

#	Step attempted (system / tool checked)	Result
1	EHR prescription transmission log.	Three prescriptions "Fax failed"; faxes to other pharmacies sent normally, so the problem is one destination.
2	E-fax server administration console, outbound queue.	Three jobs failed after three retries: "no answer / handshake failed".
3	EHR pharmacy directory fax number vs. number confirmed by the pharmacy.	Match. Not a directory error.
4	Test fax to the IT Service Desk test line.	Delivered. Ridgeline's fax server and lines work.
5	Test fax to the pharmacy; called pharmacy staff.	Failed. The pharmacy's fax line is down and its carrier is repairing it (no ETA). Cause is outside Ridgeline.

Workaround: the provider phoned all three prescriptions to the pharmacy at 10:00 AM. With the provider's approval I cancelled the failed jobs at 10:05 AM so they cannot resend as duplicates.

SLA status: at 10:12 AM I asked the clinic to tell us when the pharmacy's line is restored; clock paused with 29min remaining (due 10:41 AM). No reply by end of shift.

Recommended next step for the incoming technician: when the clinic replies, the clock resumes with 29min. Send one test fax to the pharmacy; do not resend the cancelled prescriptions. If it delivers, confirm and close. If it still fails, or failures spread to other pharmacies or providers, escalate to Pharmacy Systems by ticket with the queue logs and test results.

4. Warm-Transfer Summary

Ticket ID	**T-1043:** Infusion pump alert not clearing on unit 4B (P1, Hardware). Called Biomedical Engineering at 8:52 AM; ticket reassigned at 9:02 AM.
Baseline Evidence	**Reported:** at 7:35 AM a 4B nurse reported an infusion pump alert that would not clear. **Independently confirmed (8:48–8:51 AM, with the 4B charge nurse):** • Large-volume infusion pump, asset tag RRMC-BME-04817. • Alert returns within about 60 seconds of every acknowledgment; on-screen text and code recorded in the ticket. • Pump was in active patient use; other 4B pumps are normal. • Wireless controller console: pump online, normal signal, so not a connectivity fault. **Safety check:** at 8:51 AM the infusion was moved to a backup pump, running normally. The affected pump is tagged "Do Not Use – Biomed" and left in its alert state in the 4B clean utility room.
Documented Failure Notes	**1. 7:40 AM (on-call):** acknowledged and silenced the alert per on-screen prompts. It returned within a minute because the device itself is generating it. **2. 8:05 AM (on-call):** checked other 4B pumps; none affected, so not a unit-wide configuration push. The ticket then stalled and breached. **3. 8:50 AM (me):** wireless check showed the pump online, ruling out a network cause. **Why Tier 1 stops here:** diagnosing or repairing a medical device is outside Tier 1 scope and could erase its error log. The device was in patient use, so a phone call is required.

Receiving-Team-Ready Context	**Receiving team:** Biomedical Engineering **Specific ask:** (1) Diagnose and repair or replace pump RRMC-BME-04817. (2) Confirm 4B has enough backup pumps, or supply a loaner. (3) Post findings and an ETA in T-1043 and call the 4B charge nurse. (4) Advise whether other pumps of the same model need checking. **SLA remaining:** 0min. P1 breached (due 8:05 AM; 47min overdue at the 8:52 AM call). Patient risk mitigated by the backup pump. **Contacts:** 4B charge nurse; Tier 1 owner Dennis Lam until 11:00 AM, then the afternoon technician. **Acknowledged:** Biomed accepted at 8:52 AM; pump collected for bench testing at 9:25 AM.

5. End-of-Shift Summary

Queue state at close of shift (10:50 AM)

Open	In progress	Escalated	Fulfilled / Closed
0	2 (T-1044, T-1046; SLA paused)	1 (T-1043)	2 (T-1042, T-1045)

All five tickets were worked and documented, and none remains untouched in Open. T-1045 was Closed at 8:47 AM and T-1042 was Fulfilled at 10:34 AM; both met SLA. T-1043 was Escalated to Biomedical Engineering at 8:52 AM after its P1 SLA had already breached; the patient is on a backup pump, and the incoming technician should confirm Biomed has posted a diagnosis and that 4B has enough backup pumps. T-1044 (paused at 32min) and T-1046 (paused at 29min) remain in Continue Troubleshooting, waiting on the requester, and each clock restarts the moment a reply arrives, which is the next SLA checkpoint. Flag: escalate T-1044 to Clinical Informatics if any interface message shows Error, and do not resend the cancelled T-1046 faxes.

Submitted by: Dennis Lam **Time:** 10:55 AM